# CAPOX / XELOX

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## CAPOX / XELOX · active treatment cycle

**Active medicines:** Capecitabine + Oxaliplatin

**Emetogenic risk:** Moderate

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 16 mg orally once | Before Day 1 chemotherapy<br>Review QT risk, interactions, constipation and hepatic function. |
| Dexamethasone | 8 mg orally once | Before Day 1 chemotherapy<br>Reconcile all steroid prescriptions; do not duplicate. |
| Dexamethasone | 8 mg orally once daily | Days 2 and 3 after Day 1 chemotherapy<br>Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed. |
| Metoclopramide | 10 mg orally up to three times daily as required | Rescue only; duration to be completed by prescriber<br>Review renal function, neurological contraindications and interactions. |

### Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.
- Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.
- Where prescribed under the capecitabine source: loperamide 4 mg initially then 2 mg after each loose stool, maximum 16 mg/day; follow the written local escalation plan. Do not copy the irinotecan high-dose schedule.
- If taking warfarin: baseline and regular international normalised ratio (blood-clotting test) (INR) monitoring as clinically indicated.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00321 v9 · NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required